

HOW TO OPEN A HOMEMAKER & COMPANION SERVICES BUSINESS in Florida

An AHCA-registered non-medical provider — a light-touch, mostly private-pay business.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Florida model — read this first

Florida is one of the easiest states in the country to start a non-medical home-care business — because if you provide ONLY homemaker and companion services, you don't get a license, you just REGISTER with the Agency for Health Care Administration (AHCA) under Florida Statutes s. 400.509. There's no on-site survey, no administrator exam, and the fee is tiny. The trade-off is a hard limit on what you can do: a registered Homemaker & Companion Services (HCS) provider may not put hands on a client — no bathing, dressing, toileting, or any medical care. This guide walks the whole path — registering, then building the business — broken down so a first-timer can follow it, and it's crystal-clear about the line you must not cross.

The one line that defines this business — read it twice. A Homemaker & Companion Services registration lets you do housekeeping, meals, shopping, errands, transportation, and companionship/"sitter" supervision. It does NOT let you provide hands-on personal care (bathing, dressing, toileting, transferring) or any nursing/medical service — those require a licensed Home Health Agency (Part 2). Stay on the non-hands-on side of that line and Florida's registration is fast, cheap, and survey-free. Cross it under only an HCS registration and you risk penalties and loss of registration.

What this means for your money. Because you can't do hands-on personal care, Medicaid personal-care dollars mostly aren't open to a plain HCS registrant — this is largely a PRIVATE-PAY business (families, long-term-care insurance, and veterans' benefits). That's not a weakness: Florida's enormous senior and 55+ population makes private-pay companion and homemaker care a big, growing market. Part 8 lays out the realistic revenue.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened a business — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with AHCA and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

Nine milestones take you from idea to a registered, operating, paid business. Each part below is one rung.

1	Decide this is your business	A non-medical companion & homemaker business — recurring, hourly revenue (Part 1)
2	Know the HCS/HHA line	What you can and can't do — and when you'd need an HHA license (Part 2)
3	Form the business & insure	Entity, EIN, insurance & bonding, local business tax receipt (Part 3)
4	Register with AHCA	The s. 400.509 registration, background screening, the \$50 fee (Part 4)
5	Recruit & screen caregivers	Employment-history verification /

		background screening (Part 5)
6	Set up operations	Policies, scheduling, and your service agreement (Part 6)
7	Get paid	Private pay, LTC insurance, and veterans' benefits (Part 8)
8	Get your first clients	Discharge planners, 55+ communities, the Elder Helpline (Part 10)
9	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. This is a fast setup — form the entity, get insured, complete the background screening, and file the AHCA registration. The real work is building referral relationships (Part 10) in Florida's huge senior market. Put your AHCA registration number on every ad (the statute requires it).

PART 1

What This Business Is

In short: An AHCA-registered Homemaker & Companion Services provider — caregivers who help clients live independently at home *WITHOUT* hands-on personal care. Non-medical, private-pay, light-touch.

Florida law (s. 400.509) lets a business that provides only homemaker and companion services register with AHCA instead of getting a full license. “Homemaker services” are household tasks — light housekeeping, laundry, meal planning and preparation, shopping and errands. “Companion services” (also called “sitter” services) are non-medical supervision and social engagement — being with the client, conversation, reminders, escorting to appointments, and light help that doesn't involve physical care.

What you MAY and MAY NOT do

You MAY (registered HCS)	You MAY NOT (needs an HHA license)
Light housekeeping, laundry, meal prep	Bathing, showering, dressing, grooming with hands-on help
Grocery shopping, errands, transportation	Toileting, incontinence care, transferring/lifting
Companionship, supervision, reminders	Administering or handing medications by dose
Escorting to appointments; “sitter” care	Any nursing, wound care, or skilled/medical service

The distinction is physical, hands-on personal care and anything medical. If you never cross into those, the HCS registration is all you need.

Key terms. AHCA = Agency for Health Care Administration (registers/licenses you). HCS = Homemaker & Companion Services (your registration). HHA = Home Health Agency (the license needed for hands-on personal care).

Clearinghouse = the Care Provider Background Screening Clearinghouse. SMMC LTC = Statewide Medicaid Managed Care Long-Term Care. LTC insurance = long-term-care insurance. A&A = VA Aid & Attendance. ADRC = Aging & Disability Resource Center. Sunbiz = the Florida Division of Corporations.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. Florida has the largest 65-and-over population share in the nation and a steady stream of retirees moving in, so private-pay companion and homemaker care is one of the deepest, fastest-growing home-care markets in the country. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) *undercapitalization* — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) *weak caregiver recruiting and retention* — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) *sloppy compliance* — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

The HCS Registration vs. the HHA License — Which Path

In short: Register as HCS for non-hands-on work (fast, cheap, no survey). If you'll ever provide hands-on personal care or nursing, you instead need a licensed Home Health Agency — a heavier path. Know the line before you build.

The HCS registration (this guide's path)

Under s. 400.509, a homemaker/companion/sitter-only provider registers with AHCA's Long Term Care Services Unit. There is no on-site survey, no required administrator credential, and the fee is \$50 for a two-year (biennial) registration. It's issued in addition to the general requirements of Chapter 408, Part II (which governs background screening). You must put your AHCA registration number in any advertisement.

The Home Health Agency (HHA) license — if you want personal care

To provide hands-on personal care or nursing, you need a Home Health Agency license under s. 400.464 — a much heavier path: an on-site licensure survey, a designated Administrator and a Director of Nursing (an RN), proof of financial ability to operate, and either state survey or accreditation. Two Florida realities make this harder: AHCA has periodically imposed moratoria on new HHA licenses in certain high-density counties (to fight Medicaid fraud), and the process is far more involved. Many Florida non-medical operators deliberately stay on the HCS registration path for exactly these reasons.

A third model: the Nurse Registry

A Nurse Registry (s. 400.506) refers and places independent-contractor caregivers (aides and nurses) to clients rather than employing them. It's a different regulatory model with its own rules — mentioned here so you know it exists; this guide covers the HCS registration.

Start HCS, grow later if you want. Registering as HCS gets you earning quickly in a huge market. If demand for hands-on personal care justifies it later, you can pursue an HHA license then — after you've built referral relationships and cash flow. Confirm current HHA moratorium counties with AHCA before counting on that upgrade.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Florida you form the LLC at the Division of Corporations (“Sunbiz”) — the state’s online business-filing office at dos.fl.gov/sunbiz (help line 850-245-6051). You’ll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File Articles of Organization (LLC) on Sunbiz.** \$125 total — a \$100 filing fee plus the \$25 registered-agent designation. Name the LLC and list a registered agent (can be you).
- 2. Adopt an Operating Agreement.** The internal document that sets out ownership, management, and how money moves — Florida doesn’t file it, but your bank and partners will expect it.
- 3. Get your local business tax receipt.** Most Florida counties and cities require a local business tax receipt (formerly an “occupational license”) from the county or city tax collector before you operate.
- 4. Calendar your Florida annual report.** File it between January 1 and May 1 each year — \$138.75 — or face a \$400 late fee and possible administrative dissolution of your LLC.

No newspaper publication in Florida. *Florida has no LLC publication requirement — forming the entity is a simple Sunbiz filing plus your local business tax receipt. That’s one less cost and delay than states like New York.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business’s federal tax number — think of it as a Social Security number for the company. It’s free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here’s exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party’s legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can’t be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. For a plain Florida Homemaker & Companion registration this is OPTIONAL, because you’re a private-pay business that doesn’t bill Medicaid — but it’s free and worth getting if you later add a Medicaid contract, bill long-term-care insurers, or contract with the VA, all of which use it to identify your agency. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a major bank with lots of Florida branches — Chase, Bank of America, Wells Fargo, or Truist — or a local Florida credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

Even though there's no AHCA survey, clients and referral partners expect a professional, insured, and bonded business — a home-care operation sends people into clients' homes, which is exactly the risk these coverages are built for:

Coverage	What it protects against	Must-have?
General Liability (GL)	Bodily injury or property damage in a client's home	Yes — foundational
Professional Liability	Claims about the service itself — a caregiver's error or neglect	Yes
Abuse & Molestation	Allegations of abuse by a caregiver — GL EXCLUDES this	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home by an employee — families ask if you're “bonded”	Yes — clients ask if you're bonded
Non-Owned & Hired Auto	Caregivers driving their own cars for or with clients	Yes — critical for home care
Workers' Compensation	Caregiver injuries on the job	Required in FL at 4+ employees (non-construction)

Real places to get it

Buy through an independent agent or broker who writes home-care programs — carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a Florida agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care). Confirm the carrier writes non-medical home care in Florida before you rely on a quote.

What to ask the insurance agent

“Do you write non-medical homemaker/companion agencies — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, and at what limit? Is non-owned & hired auto included?”

“Do you include an employee-dishonesty bond, and at what amount?”

“When does Florida require me to carry workers' comp, given my number of caregivers?”

PART 4

Register with AHCA — Step by Step

In short: Complete background screening, fill out the AHCA Homemaker & Companion Services application, pay the \$50 biennial fee, and you're registered — no survey. Then keep the registration current and put its number on your ads.

4.1 Before you register

- ☐ Entity formed on Sunbiz; EIN obtained; local business tax receipt
- ☐ Liability insurance bound; an employee-dishonesty bond arranged
- ☐ Background screening completed for the owner/registrant (Part 5)
- ☐ Your service policies and a client service agreement ready

4.2 The registration

- 1. Get the application.** AHCA Form 3110-1003 (Health Care Licensing Application — Homemaker and Companion Services Provider), which references Rule 59A-35.060, F.A.C. From the AHCA Long Term Care Services Unit.
- 2. Complete the background screening.** Owners and covered persons complete the screening AHCA requires through the Care Provider Background Screening Clearinghouse (Part 5) — confirm with AHCA which level applies to a companion-only registrant.
- 3. Pay the fee.** \$50 per biennium (a two-year registration).
- 4. Submit to AHCA's Long Term Care Services Unit.** Phone (850) 412-4303; email LTCStaff@ahca.myflorida.com. There is NO on-site survey.
- 5. Put your registration number on every ad.** The statute requires your AHCA registration number in any advertisement for your services.

Renew on time. The registration is valid for up to two years and renews on re-verification. Calendar the renewal — an expired registration means you're operating unregistered, which carries penalties.

4.3 Growing later — the private-pay-to-Home-Health path

Many Florida owners start exactly here — a light-touch HCS registration on private pay — and use it as a launchpad. Once you've built a caregiver roster, referral relationships, and steady cash flow, the natural next step is a Home Health Agency (HHA) license under s. 400.464, which unlocks hands-on personal care, VA and long-term-care contracts that require a licensed provider, and Medicaid personal-care dollars. It's a heavier lift, so plan it deliberately rather than stumbling into it:

- 1. Confirm the county isn't under a moratorium.** AHCA has periodically frozen new HHA licenses in certain high-density counties to fight Medicaid fraud. Before you invest a dollar in the upgrade, call AHCA and confirm your county is open to new HHA applications.
- 2. Build the required leadership.** An HHA license requires a designated Administrator and a Director of Nursing (an RN). Line up these people — or a plan to hire them — well before you apply, because they anchor the whole application.
- 3. Prepare for the survey and financial proof.** Unlike the survey-free HCS registration, an HHA license means an on-site licensure survey (or accreditation in lieu of it) and proof of financial ability to operate. Budget several months and real capital.
- 4. Decide on Medicaid enrollment.** If you want SMMC Long-Term-Care volume, you'll enroll with AHCA and contract with the managed-care plans — and take on Electronic Visit Verification for those visits, which private-pay work never requires.

Why staying HCS a while is smart. Starting on the HCS registration lets you earn immediately in a huge market with almost no regulatory friction, and it's the low-risk way to learn the business. When — and only when —

demand for hands-on care and your cash position justify it, the HHA license is there. Confirm current moratorium counties with AHCA before you count on the upgrade.

PART 5

Your Caregivers & Background Screening

In short: Florida requires you to screen the people who go into clients' homes. Get this right — it's your biggest liability protection and a top question every referral source and family will ask.

5.1 Background screening — confirm your level with AHCA

Florida's health-care background screening comes in levels. Level 2 is a fingerprint-based state (FDLE) and national (FBI) check run through the Care Provider Background Screening Clearinghouse. For a licensed provider (like an HHA) doing personal care, Level 2 is clearly required. For a companion-only HCS registrant, the registration statute (s. 400.509) specifically requires you to obtain and verify each caregiver's employment/contract history — and it's not fully settled in every case whether pure non-hands-on registrants must also complete Level 2 fingerprinting.

Do this: confirm your screening level with AHCA before you hire. Call AHCA's Long Term Care Services Unit (850-412-4303) and ask exactly which screening applies to a Homemaker & Companion Services registrant — employment-history verification, Level 2 Clearinghouse fingerprinting, or both. Then screen to that standard for every caregiver, and keep the records. Many operators run Level 2 anyway because families and referral partners expect it.

5.2 Level 2 screening — how it works (if it applies)

- **Where:** the Care Provider Background Screening Clearinghouse (info.flclearinghouse.com), with Livescan electronic fingerprints submitted to FDLE and the FBI.
- **Fees:** new applicant screening \$60 plus the Livescan vendor's fee (varies), renewal \$42, resubmission \$12 — confirm current amounts.
- **Timing:** fingerprints reach FDLE in about 1–3 days; the full review typically takes about 5–7 business days.

5.3 Hiring & keeping good caregivers

You employ your caregivers (W-2), so you handle payroll, scheduling, and supervision. Beyond screening, invest in a solid orientation, a clear scope-of-work (so caregivers know NOT to do hands-on personal care), and consistent scheduling — caregiver quality and reliability are the number-one driver of client satisfaction and referrals. Give every caregiver written boundaries on the HCS/HHA line so no one drifts into hands-on care.

5.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

Policies & Getting Operational

In short: Write your policies and your service agreement, set up scheduling and invoicing, and make the HCS scope crystal-clear to clients and staff. This is a light-touch model — keep your operations clean and professional.

6.1 Your policies & service agreement

Even without a survey, run a professional operation: a service agreement that states clearly what you do and don't provide (non-medical, non-hands-on), the schedule and rate, client rights and confidentiality, complaint handling, and how service can end. Add policies for scheduling, caregiver conduct, incident reporting, and emergency procedures. Clear scope language protects you if a family ever asks for hands-on care you're not registered to provide.

6.2 Set up your systems

Choose a home-care software platform for scheduling, caregiver and client records, visit tracking, and invoicing. Since you're private-pay, you won't need Medicaid EVV. Set up simple, reliable billing (many private-pay agencies bill weekly or bi-weekly, or take a card on file).

6.3 Handling a request that crosses the line

Families will sometimes ask your companion to “just help mom into the shower.” Train staff to say no and to route it: either the client arranges a licensed home-health aide for the hands-on task, or you refer to a partner HHA. Document these boundaries. Protecting the line protects your registration.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Inquiry to Discharge

In short: A client is referred (usually privately), you assess the need and confirm it's non-medical, you match a caregiver and start service, you supervise and adjust, and you transition well. Here is the whole journey, stage by stage.

Stage 1 — Inquiry & referral

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, a 55+ community, or the Elder Helpline (Part 10). Someone asks whether you can help a specific person at home.

What you do:

- Take the basics (who, what tasks, how many hours, where, when to start, who pays). Confirm the need is non-medical, non-hands-on — if they need bathing/dressing/toileting help, refer to an HHA or arrange it separately.

Stage 2 — In-home consultation & the service plan

Visit the home, understand the routine and the household, and build a simple service plan: the tasks (housekeeping, meals, errands, transport, companionship), the schedule and hours, and any safety notes. Set expectations about scope.

What you do:

- Walk the home and note fall risks, pets, parking, and door codes; list the exact tasks and hours; write the scope in plain language so the family sees what is and isn't included; and quote the rate and any visit minimum in writing before anyone signs.

Stage 3 — Agreement & start of care

The client (or family) signs your service agreement — services, schedule, rate, scope, rights, and termination. Match a screened caregiver whose personality and skills fit the client, and make the first visit a warm, supervised introduction.

What you do:

- Confirm payment method (card on file, LTC-insurance claim, or VA arrangement) before the first shift; introduce the caregiver in person; leave a schedule and your contact number with the family; and set the date for your first check-in call.

Stage 4 — Delivering the service

The caregiver delivers the scheduled help; keep light visit notes and protect the client-caregiver match. Consistency of caregiver is the number-one driver of satisfaction and retention.

Stage 5 — Check-ins & adjustments

Check in with the client and family on a schedule, adjust hours and tasks as life changes, and watch for signs the client now needs hands-on personal care (a cue to bring in an HHA partner). Handle complaints fast.

What you do:

- Call the family on a set cadence (weekly at first), log any change in the client's needs, adjust the schedule in writing, and act on the first sign that hands-on care is needed — that's the moment to bring in a licensed HHA partner rather than let a companion drift over the line.

Stage 6 — Transition

When needs change or service ends, transition gracefully — refer to a higher level of care if needed, give notice per your agreement, and follow up. A good ending turns families into referral sources.

What you do:

- Give written notice per the agreement, hand off cleanly to whatever comes next (an HHA, a facility, or hospice), return any keys or paperwork, and follow up with a thank-you — the families you treat well at the end are the ones who refer you their neighbors.

The whole journey stays non-medical. At every stage above, your job is homemaker and companion help — housekeeping, meals, errands, transport, and companionship. The instant a stage reveals a need for hands-on personal care or anything medical, that's a referral to an HHA, not a task for your caregiver. Keeping that discipline through the whole client journey is what protects your registration.

PART 8

How You Get Paid — Private Pay, LTC Insurance & Veterans

In short: A registered HCS provider is a private-pay business. Medicaid personal-care dollars mostly require a licensed provider — so build on private pay, long-term-care insurance, and veterans' benefits.

Your revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Your core market — Florida's huge senior population; ~\$30/hr (2024)
Long-term-care insurance	The client's LTC policy reimburses home care	Many policies cover homemaker/companion; bill the family or insurer
Veterans (VA)	VA benefits, esp. Aid & Attendance	A&A cash lets veterans afford private-pay companion care
Medicaid (SMMC LTC)	Statewide Medicaid Managed Care – Long-Term Care	Mostly needs a licensed provider; a plan may contract non-medical work — verify

8.1 Private pay — your core market

Florida's enormous population of seniors and 55+ residents makes private-pay companion and homemaker care a large market. You bill hourly (often with a visit minimum), typically around \$30/hour in Florida (2024 cost-of-care data; verify the current figure) — set your rate above your fully-loaded caregiver cost. Families pay directly, weekly or via card on file.

8.2 Long-term-care insurance

Many long-term-care insurance policies reimburse non-medical homemaker/companion and personal care once the benefit triggers are met (typically needing help with two or more ADLs, or cognitive impairment). Some policies require the provider to be AHCA-registered or licensed and caregivers to be background-screened — being a registered HCS provider positions you to serve these clients. You bill the family or the insurer and help with the claim; keep clear visit records.

8.3 Veterans

- **Aid & Attendance (A&A):** an enhanced VA pension for eligible wartime veterans/survivors — extra monthly cash the veteran can spend on in-home care, including non-medical companion/homemaker help. This effectively funds private-pay care and is a big deal in Florida. (2025–2026 maximums are up to about \$2,424/month for a single veteran, \$1,558 for a surviving spouse — verify on VA.gov.)
- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** VA-arranged in-home programs; the hands-on personal-care portion generally needs a contracted (often HHA-level) provider, but companion/homemaker help fits the homemaker portion. Florida is in VA Community Care Network Region 3, administered by Optum (Community Care line 844-839-6120).

8.4 A note on Medicaid

Florida's Statewide Medicaid Managed Care Long-Term Care program (SMMC — the current “SMMC 3.0” contracts took effect February 1, 2025) covers homemaker, companion, personal care, and attendant care — but delivered through AHCA-enrolled providers contracted with the plans. The statewide LTC plans are Humana Healthy Horizons, Sunshine Health, Simply Healthcare, and Florida Community Care (with additional regional plans). Personal-care/attendant dollars generally require a licensed provider; some plans may contract for non-medical homemaker/companion work with a registered provider, but that's plan-by-plan — treat any Medicaid contract as an upside to negotiate, not your base model. (Florida's consumer-directed CDC+ option, run by the Agency for Persons with Disabilities, lets certain waiver participants hire their own workers.)

PART 9

What It Costs & How Long It Takes

In short: This is a low-cost, fast setup — the biggest investments are your time building referrals and your caregiver payroll, not licensing.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
AHCA registration	HCS registration (2-year)	\$50 / biennium
Background screening	Per person, if Level 2 applies	~\$60 + Livescan fee each (verify level)
Business setup	Sunbiz LLC \$125 + \$138.75/yr report, EIN free, business tax receipt, legal	~\$300 + legal
Insurance & bond	GL + professional + abuse + non-owned auto + dishonesty bond	Get quotes — annual
Software	Scheduling / records / invoicing	Monthly SaaS
Payroll float	You pay caregivers before some clients/insurers pay	Working capital
Policies	P&P + service agreement + forms	One-time
Marketing	One-pagers, website, referral outreach	Ongoing — your biggest growth lever

9.2 Timeline

Week 1	Form the business	Sunbiz LLC, EIN, business tax receipt
Weeks 1-2	Insure & screen	Bind coverage & bond; complete background screening
Weeks 2-3	Register with AHCA	File Form 3110-1003 + \$50 — no survey to schedule
Right away	Start private pay	Serve private-pay & LTC-insurance clients
Ongoing	Build referrals	Discharge planners, 55+ communities, the aging network

No accreditation needed. AHCA does not require accreditation to register as a homemaker/companion provider (there's no survey to waive). Accreditation only becomes relevant if you later upgrade to a licensed Home Health Agency.

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$30/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

The AHCA registration is cheap, so first-timers often under-budget the things that actually cost money. Plan for these before you open:

- **Payroll before you're paid:** you pay caregivers weekly, but a family on a card-on-file or an LTC insurer may pay you slower — carry several weeks of payroll as working capital.
- **Background screening per caregiver:** if Level 2 applies, it's roughly \$60 plus the Livescan vendor fee for each hire, and it repeats as you grow and re-screen.
- **Non-owned auto and mileage:** caregivers drive clients and run errands — the auto coverage and any mileage reimbursement are real, recurring costs, not one-time ones.
- **Turnover:** every caregiver who quits costs you re-recruiting, re-screening, orientation, and sometimes the client they served — retention is a cost-control strategy, not just an HR nicety.
- **Owner's own pay:** budget for the months before the business can pay you a salary; under-capitalization here sinks more agencies than any regulatory misstep.
- **Software, phones, and marketing:** the monthly scheduling/records platform, a business phone line, and the one-pagers and website that feed your referral pipeline all recur every month.

PART 10

Getting Your First Clients

In short: In Florida's huge senior market, referral relationships are everything. Discharge planners, home-health/hospice partners, 55+ communities, and the aging network are your pipeline. Build them before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the highest-volume source for “needs help at home now” — introduce yourself, leave a one-pager, and make same-day starts easy.
- **Skilled home-health & hospice partners:** they provide the intermittent skilled care; you fill the non-medical companion/homemaker hours Medicare doesn't cover. Build reciprocal referrals.
- **55+, active-adult & CCRC communities:** Florida has more of these than anywhere — build preferred-provider relationships with residence directors and activity staff.
- **The aging network:** Florida's Elder Helpline (1-800-963-5337) routes callers to local agencies, and the 11 Area Agencies on Aging / Aging & Disability Resource Centers and the SHINE Medicare-counseling program are trusted referral hubs — get listed and known.
- **Geriatric care managers & elder-law attorneys:** Aging Life Care professionals place clients with vetted agencies, and elder-law/financial-planning professionals (especially VA-accredited claims agents tied to Aid & Attendance) refer families.
- **2-1-1 & senior expos:** list with 2-1-1, and show up at Florida's many senior expos and community events.

10.2 What to ask each referral source

“How do you decide which home-care agency to refer a family to?”

“What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?”

“Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?”

“How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?”

“What makes an agency easy to work with, and what gets one dropped from your list?”

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Registration, screening & the scope line

Who	Contact	What to verify / ask
AHCA — Long Term Care Services Unit	(850) 412-4303 · LTCStaff@ahca.myflorida.com	The HCS registration (Form 3110-1003, \$50); the operative rule cite; which background-screening level applies
AHCA — Home Health Agency licensing	ahca.myflorida.com (Health Facility Regulation)	If you'll do personal care: HHA license requirements; current moratorium counties
Background Screening Clearinghouse	info.flclearinghouse.com	Level 2 screening steps and current fees; whether it applies to you
City/County tax collector	your jurisdiction	Your local business tax receipt

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Florida Division of Corporations (Sunbiz)	850-245-6051 · dos.fl.gov/sunbiz	Forming your LLC (\$125); the \$138.75 annual report
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
Insurance agent (home-care program)	trustedchoice.com; carriers in Part 3.4	GL + abuse + non-owned auto + dishonesty bond; when workers' comp is required
Business bank	Chase/BofA/Wells/Truist; a local credit union	Account fees; a business line of credit for payroll

Funding & referral partners

Who	Contact	What to verify / ask
Florida Elder Helpline (Dept. of Elder Affairs)	1-800-963-5337 · elderaffairs.org	Getting listed; the local Area Agency on Aging / ADRC for your area
SMMC LTC plans	Humana, Sunshine, Simply, Florida Community Care (Part 8.4)	Whether they contract non-medical companion/homemaker work in your region
VA Community Care / Optum (Region 3)	844-839-6120 · va.gov/communitycare	The VA Homemaker/HHA program; Aid & Attendance for your clients
Long-term-care insurers	the client's policy / carrier	Benefit triggers; whether an AHCA registration is required; how to bill

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

You can do this

Opening a Florida homemaker & companion business is one of the fastest, lowest-cost ways to enter home care — a simple AHCA registration, no survey, and one of the biggest senior markets in the country. Take it one rung at a time: form the business, insure and bond it, complete your background screening, register with AHCA, and pour your energy into referral relationships. Stay firmly on the non-hands-on side of the HCS line, verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in Florida Statutes s. 400.509 and current AHCA information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with AHCA and qualified counsel before you rely on it. In particular, confirm with AHCA which background-screening level applies to your registration, and never provide hands-on personal care without the proper Home Health Agency license.